

1. NAME OF THE MEDICINAL PRODUCT

Pantoprazole Sandoz 40 mg powder for solution for injection

2. QUALITATIVE AND QUANTITATIVE COMPOSITION

Each vial contains 40 mg of pantoprazole (as sodium sesquihydrate).

For a full list of excipients, see section 6.1.

3. PHARMACEUTICAL FORM

White to yellowish powder for solution for injection.

After reconstitution, the solution is colourless to faintly yellow.

4. CLINICAL PARTICULARS

4.1 Therapeutic indications

Short term use for symptomatic improvement and healing of gastrointestinal diseases which require a reduction in acid secretion:

- Moderate and severe reflux oesophagitis
- Gastric and duodenal ulcer
- Zollinger-Ellison-Syndrome and other pathological hypersecretory conditions

4.2 Posology and method of administration

This medicine should be administered by a healthcare professional and under appropriate medical supervision.

The intravenous administration of Pantoprazole Sandoz 40 mg powder for solution for injection is recommended only if oral application is not appropriate. The recommended dose for gastric and duodenal ulcer and moderate and severe reflux esophagitis is one vial of 40 mg Pantoprazole Sandoz per day.

For the long-term management of Zollinger-Ellison-Syndrome and other pathological hypersecretory conditions, the recommended daily dose at the beginning of treatment is 80 mg pantoprazole. Thereafter, the dosage can be titrated up or down as needed using measurements of gastric acid secretion to guide. With doses above 80 mg daily, the dose should be divided and given twice daily. A temporary increase of the dosage above 160 mg pantoprazole is possible but should not be applied longer than required for adequate acid control. In case a rapid acid control is required, a starting dose of 2 x 80 mg pantoprazole is sufficient to manage a decrease of acid output into the target range (<10 mEq/h) within one hour in the majority of patients.

Transition from Pantoprazole Sandoz 40 mg powder for solution for injection to the oral formulation of Pantoprazole should be performed as soon as it is clinically justified.

Instruction for use/ handling

A ready-to-use solution is prepared by injecting 10 ml of physiological sodium chloride solution into the vial containing the dry substance. For instructions for preparation, see section 6.5.

The solution may be administered directly or may be administered after mixing with 100 ml physiological sodium chloride solution or 5% glucose.

Pantoprazole Sandoz should not be prepared or mixed with solvents other than those stated.

After preparation, the solution must be used within 12 hours. From a microbiological point of view, the product should be used immediately. If not used immediately, in-use storage times and conditions prior to use are the responsibility of the user and would normally not be longer than 12 hours at not more than 25°C.

The medicinal product should be administered intravenously over 2 -15 minutes.

Special Patient Populations

Impaired hepatic function

A daily dose of 20 mg pantoprazole (half a vial of 40 mg pantoprazole) should not be exceeded in patients with severe liver impairment (see section 4.4).

Impaired renal function

No dose adjustment is necessary in patients with impaired renal function (see section 5.2).

Type and duration of treatment

As soon as oral therapy is possible, treatment with Pantoprazole Sandoz 40 mg powder for solution for injection should be discontinued and 40 mg pantoprazole p.o. should be administered instead.

4.3 Contraindications

Hypersensitivity to the active substance, substituted benzimidazoles, or to any of the other excipients.

4.4 Special warnings and special precautions for use

Regular Surveillance

Patients on proton pump inhibitor treatment (particularly those treated for long term) should be kept under regular surveillance.

Gastric malignancy

Symptomatic response to pantoprazole may mask the symptoms of gastric malignancy and may delay diagnosis. In the presence of any alarm symptom (e.g. significant unintentional weight loss, recurrent vomiting, dysphagia, haematemesis, anaemia or melaena) and when gastric ulcer is suspected or present, malignancy should be excluded.

Further investigation is to be considered if symptoms persist despite adequate treatment.

Hepatic Impairment

In patients with severe liver impairment, the liver enzymes should be monitored during therapy, particularly on long-term use. In the case of arise of the liver enzymes, the treatment should be discontinued (see section 4.2).

Co-administration with HIV protease inhibitors

Co-administration of pantoprazole is not recommended with HIV protease inhibitors for which absorption is dependent on acidic intragastric pH such as atazanavir, nelfinavir; due to significant reduction in their bioavailability (see section 4.5).

Gastrointestinal infections caused by bacteria

Treatment with pantoprazole may lead to a slightly increased risk of gastrointestinal infections caused by bacteria such as *Salmonella*, *Campylobacter* and *C. difficile*.

Clostridium Difficile Diarrhoea

Published observational studies suggest that PPI therapy may be associated with an increased risk of *Clostridium difficile* associated diarrhoea, especially in hospitalized patients. This diagnosis should be considered for diarrhoea that does not improve. Patients should use the lowest dose and shortest duration of PPI therapy appropriate to the condition being treated.

Hypomagnesaemia

Severe hypomagnesaemia has been reported in patients treated with PPIs like pantoprazole for at least three months, and in most cases for a year. Serious manifestations of hypomagnesaemia such as fatigue, tetany, delirium, convulsions, dizziness and ventricular arrhythmia can occur but they may begin insidiously and be overlooked. In most affected patients, hypomagnesaemia improved after magnesium replacement and discontinuation of the PPI.

For patients expected to be on prolonged treatment or who take PPIs with medications such as digoxin or drugs that may cause hypomagnesaemia (e.g., diuretics), health care professionals should consider measuring magnesium levels before starting PPI treatment and periodically during treatment.

Bone fractures

Proton pump inhibitors, especially if used in high doses and over long durations (>1 year), may modestly increase the risk of hip, wrist and spine fracture, predominantly in the elderly or in presence of other recognised risk factors. Observational studies suggest that proton pump inhibitors may increase the overall risk of fracture by 10-40%. Some of this increase may be due to other risk factors. Patients at risk of osteoporosis should receive care according to current clinical guidelines and they should have an adequate intake of vitamin D and calcium.

Subacute cutaneous lupus erythematosus (SCLE)

Proton pump inhibitors are associated with very infrequent cases of SCLE. If lesions occur, especially in sun-exposed areas of the skin, and if accompanied by arthralgia, the patient should seek medical help promptly and the health care professional should consider stopping Pantoprazole 40 mg Powder for Solution for Injection. SCLE after previous treatment with a proton pump inhibitor may increase the risk of SCLE with other proton pump inhibitors.

Interference with laboratory tests

Increased Chromogranin A (CgA) level may interfere with investigations for neuroendocrine tumours. If the patient(s) are due to have a test on Chromogranin A level, Pantoprazole Sandoz treatment should be stopped for at least 5 days before CgA measurements to avoid this interference (see section 5.1 Pharmacodynamics). If CgA and gastrin levels have not returned to reference range after initial measurement, measurements should be repeated 14 days after cessation of proton pump inhibitor treatment.

Vitamin B12 Deficiency

Daily treatment with any acid-suppressing medications over a long period of time (e.g. longer than 3 years) may lead to malabsorption of cyanocobalamin (vitamin B12) caused by hypo- or achlorhydria. Rare reports of cyanocobalamin deficiency occurring with acid-suppressing therapy have been reported in the literature. This diagnosis should be considered if clinical symptoms consistent with cyanocobalamin deficiency is observed.

Methotrexate

Concomitant use with high dose methotrexate may elevate and prolong serum levels of methotrexate and/or its metabolite, possibly leading to methotrexate toxicities.

Severe cutaneous adverse reactions

Severe cutaneous adverse reactions, including erythema multiforme, Stevens-Johnson syndrome (SJS), toxic epidermal necrolysis (TEN), drug reaction with eosinophilia and systemic symptoms (DRESS), and acute generalized exanthematous pustulosis (AGEP) have been reported in association with the use of PPIs (see section 4.8). Discontinue pantoprazole at the first signs or symptoms of severe cutaneous adverse reactions or other signs of hypersensitivity and consider further evaluation.

4.5 Interactions with other medicinal products and other forms of interaction

Medicinal products with pH dependent absorption pharmacokinetics

Because of profound and long lasting inhibition of gastric acid secretion, pantoprazole may interfere with the absorption of medicinal products where gastric pH is an important determinant of oral bioavailability, e.g. some azole antifungals such as ketoconazole, itraconazole, posaconazole and other medicine such as erlotinib.

HIV protease inhibitors

Co-administration of pantoprazole is not recommended with HIV protease inhibitors for which absorption is dependent on acidic intragastric pH such as atazanavir due to significant reduction in their bioavailability (see section 4.4).

If the combination of HIV protease inhibitors with a proton pump inhibitor is judged unavoidable, close clinical monitoring (e.g. virus load) is recommended. A pantoprazole dose of 20 mg per day should not be exceeded. Dose of the HIV protease inhibitor may need to be adjusted.

Coumarin anticoagulants (phenprocoumon or warfarin)

Co-administration of pantoprazole with warfarin or phenprocoumon did not affect the pharmacokinetics of warfarin, phenprocoumon or INR. However, there have been reports of increased INR and prothrombin time in patients receiving PPIs and warfarin or phenprocoumon concomitantly. Increases in INR and prothrombin time may lead to abnormal bleeding, and even death. Patients treated with pantoprazole and warfarin or phenprocoumon may need to be monitored for increase in INR and prothrombin time.

Methotrexate

Concomitant use of high dose methotrexate (e.g. 300 mg) and proton-pump inhibitors has been reported to increase methotrexate levels in some patients. Therefore in settings where high-dose methotrexate is used, for example cancer and psoriasis, a temporary withdrawal of pantoprazole may need to be considered.

Other interactions studies

Pantoprazole is extensively metabolized in the liver via the cytochrome P450 enzyme system. The main metabolic pathway is demethylation by CYP2C19 and other metabolic pathways include oxidation by CYP3A4. Interaction studies with medicinal products also metabolized with these pathways, like carbamazepine, diazepam, glibenclamide, nifedipine, and an oral contraceptive containing levonorgestrel and ethinyl oestradiol did not reveal clinically significant interactions.

An interaction of pantoprazole with other medicinal products or compounds, which are metabolised using the same enzyme system, cannot be excluded.

Results from a range of interaction studies demonstrate that pantoprazole does not affect the metabolism of active substances metabolised by CYP1A2 (such as caffeine, theophylline), CYP2C9 (such as piroxicam, diclofenac, naproxen), CYP2D6 (such as metoprolol), CYP2E1 (such as ethanol) or does not interfere with p-glycoprotein related absorption of digoxin.

There were no interactions with concomitantly administered antacids.

Interaction studies have also been performed administering pantoprazole concomitantly with the respective antibiotics (clarithromycin, metronidazole, amoxicillin). No clinically relevant interactions were found.

Clopidogrel

Concomitant administration of pantoprazole and clopidogrel in healthy subjects had no clinically important effect on exposure to the active metabolite of clopidogrel or clopidogrel-induced platelet inhibition. No dose adjustment of clopidogrel is necessary when administered with an approved dose of pantoprazole.

Medicinal products that inhibit or induce CYP2C19 (tacrolimus, fluvoxamine)

Concomitant administration of pantoprazole and tacrolimus may increase whole blood levels of tacrolimus, especially in transplant patients who are intermediate or poor metabolizers of CYP2C19. Inhibitors of CYP2C19, such as fluvoxamine, would likely increase the systemic exposure of pantoprazole.

Coumarin anticoagulants (phenprocoumon or warfarin)

Co-administration of pantoprazole with warfarin or phenprocoumon did not affect the pharmacokinetics of warfarin, phenprocoumon or INR. However, there have been reports of increased INR and prothrombin time in patients receiving PPIs and warfarin or phenprocoumon concomitantly. Increases in INR and prothrombin time may lead to abnormal bleeding, and even death. Patients treated with pantoprazole and warfarin or phenprocoumon may need to be monitored for increase in INR and prothrombin time.

4.6 Pregnancy and Lactation

Pregnancy

There are no adequate data from the use of pantoprazole in pregnant women. Studies in animals have shown reproductive toxicity. The potential risk for humans is unknown. Pantoprazole Sandoz 40 mg powder for solution for injection should not be used during pregnancy unless clearly necessary.

Breast-feeding

Animal studies have shown excretion of pantoprazole in breast milk.

There is insufficient information on the excretion of pantoprazole in human milk but excretion into human milk has been reported.

A risk to the newborns/infants cannot be excluded.

Therefore, a decision on whether to continue/discontinue breast-feeding or to continue/discontinue therapy with Pantoprazole Sandoz 40 mg powder for solution for injection should be made taking into account the benefit of breast-feeding to the child and the benefit of Pantoprazole Sandoz 40 mg powder for solution for injection therapy to women.

Fertility

There was no evidence of impaired fertility following the administration of pantoprazole in animal studies.

4.7 Effects on ability to drive and use machines

Pantoprazole has no or negligible influence on the ability to drive and use machines.

Adverse drug reactions such as dizziness and visual disturbances may occur (see section 4.8). If affected, patients should not drive or operate machines.

4.8 Undesirable effects

The table below lists adverse reactions reported with pantoprazole, ranked under the following frequency classification:

Very common ($\geq 1/10$); common ($\geq 1/100$ to $< 1/10$); uncommon ($\geq 1/1000$ to $< 1/100$); rare ($\geq 1/10,000$ to $< 1/1,000$); very rare ($< 1/10,000$), not known (cannot be estimated from the available data).

For all adverse reactions reported from post-marketing experience, it is not possible to apply any Adverse Reaction frequency and therefore they are mentioned with a “not known” frequency.

Blood and lymphatic system disorders

Rare: Agranulocytosis
Very rare: Thrombocytopenia, leukopenia, pancytopenia

Immune system disorders

Rare: Hypersensitivity (including anaphylactic reactions and anaphylactic shock)

Metabolism and nutrition disorders

Rare: Hyperlipidaemias and lipid increases (triglycerides, cholesterol); weight changes
Not known: Hyponatraemia, hypomagnesaemia (see section 4.4), hypocalcaemia in association with hypomagnesaemia, hypokalaemia, Vitamin B₁₂ deficiency

Psychiatric disorders

Uncommon: Sleep disorders
Rare: Depression (and all aggravations)
Very rare: Disorientation (and all aggravations)
Not known: Hallucination; confusion (especially in pre-disposed patients, as well as the aggravation of these symptoms in case of pre-existence)

Nervous system disorders

Uncommon: Headache, dizziness
Rare: Taste disorders

Eye disorders

Rare: Disturbances in vision/blurred vision

Gastrointestinal disorders

Common: Fundic gland polyps (benign)
Uncommon: Diarrhoea; nausea/vomiting; abdominal distension and bloating; constipation; dry mouth; abdominal pain and discomfort
Not known: Microscopic colitis

Hepatobiliary disorders

Uncommon: Liver enzymes increased (transaminases, γ -GT)
Rare: Bilirubin increased
Not known: Hepatocellular injury; jaundice; hepatocellular failure

Skin and subcutaneous tissue disorders

Uncommon: Rash/exanthema/eruption; pruritus

Rare: Urticaria; angioedema
Not known: Stevens-Johnson syndrome; Toxic epidermal necrolysis; Drug reaction with eosinophilia and systemic symptoms; Acute generalized exanthematous pustulosis; Lyell syndrome; erythema multiforme; photosensitivity; subacute cutaneous lupus erythematosus (SCLE) (see section 4.4)

Musculoskeletal and connective tissue disorders

Uncommon: Fracture of the hip, wrist or spine (see section 4.4)
Rare: Arthralgia; myalgia

Renal and urinary disorders

Not known: Interstitial nephritis (with possible progression to renal failure)

Reproductive system and breast disorders

Rare: Gynaecomastia

General disorders and administration site conditions

Common: Injection site thrombophlebitis
Uncommon: Asthenia, fatigue and malaise
Rare: Body temperature increased; oedema peripheral

Infections and Infestations

Not known: *Clostridium difficile* associated diarrhoea

4.9 Overdose

There are no known symptoms of overdose in man.

Systemic exposure with up to 240 mg administered intravenously over 2 minutes was well tolerated.

As pantoprazole is extensively protein bound, it is not readily dialysable.

In the case of overdose with clinical signs of intoxication, apart from symptomatic and supportive treatment, no specific therapeutic recommendations can be made.

5. PHARMACOLOGICAL PROPERTIES

5.1 Pharmacodynamic properties

Pharmacotherapeutic group: Proton pump inhibitors, ATC code: A02BC02.

Mechanism of action

Pantoprazole is a substituted benzimidazole which inhibits the secretion of hydrochloric acid in the stomach by specific blockade of the proton pumps of the parietal cells.

Pantoprazole is converted to its active form in the acidic environment in the parietal cells where it inhibits the H⁺, K⁺-ATPase enzyme, i.e. the final stage in the production of hydrochloric acid in the stomach. The inhibition is dose-dependent and affects both basal and stimulated acid secretion. In most patients, freedom from symptoms is achieved within 2 weeks. As with other proton pump inhibitors and H₂ receptor inhibitors, treatment with pantoprazole reduces acidity in the stomach and thereby increases gastrin in proportion to the reduction in acidity. The increase in gastrin is reversible. Since pantoprazole binds to the enzyme distal to the cell receptor level, it can inhibit hydrochloric acid secretion independently of stimulation by other substances (acetylcholine, histamine, gastrin). The effect is the same whether the product is given orally or intravenously.

Pharmacodynamic effects

The fasting gastrin values increase under pantoprazole. On short-term use, in most cases they do not exceed the upper limit of normal. During long-term treatment, gastrin levels double in most cases. An excessive increase, however, occurs only in isolated cases. As a result, a mild to moderate increase in the number of specific endocrine (ECL) cells in the stomach is observed in a minority of cases during long-term treatment (simple to adenomatoid hyperplasia). However, according to the studies conducted so far, the formation of carcinoid precursors (atypical hyperplasia) or gastric carcinoids as were found in animal experiments have not been observed in humans.

An influence of a long term treatment with pantoprazole exceeding one year cannot be completely ruled out on endocrine parameters of the thyroid according to results in animal studies.

During treatment with antisecretory medicinal products, serum gastrin increases in response to the decreased acid secretion. Also CgA increases due to decreased gastric acidity. The increased CgA level may interfere with investigations for neuroendocrine tumours.

Available published evidence suggests that proton pump inhibitors should be discontinued between 5 days and 2 weeks prior to CgA measurements. This is to allow CgA levels that might be spuriously elevated following PPI treatment to return to reference range.

5.2 Pharmacokinetic properties

General pharmacokinetics

Pharmacokinetics do not vary after single or repeated administration. In the dose range of 10 to 80 mg, the plasma kinetics of pantoprazole are linear after both oral and intravenous administration.

Distribution

Pantoprazole's serum protein binding is about 98%. Volume of distribution is about 0.15 l/kg.

Metabolism

Pantoprazole is rapidly eliminated from the circulation and extensively metabolized in the liver. Metabolism occurs via oxidation by the CYP enzyme system, predominantly by CYP2C19 and CYP3A4 (Phase I metabolism, which is saturable). Pantoprazole undergoes further biotransformation by conjugation with sulphate, which involves the cytoplasmic enzyme sulphotransferase (phase II metabolism, which is not saturable), and which presents the main metabolism of pantoprazole.

Excretion and Elimination

Renal elimination represents the major route of excretion (about 80%) for the metabolites of pantoprazole, the rest is excreted with the faeces. The main metabolite in both the serum and urine is desmethylpantoprazole which is conjugated with sulphate. The half-life of the main metabolite is about 1.5 hours (which is not much longer than that of pantoprazole, 1 hour).

Special populations

Renal impairment

In patients with impaired renal function (including dialysis), pantoprazole showed no prolonged elimination half-life and no accumulation when compared with healthy subjects. No dose adjustment is necessary in patients with impaired renal function.

Hepatic impairment

In comparison with healthy subjects, after oral administration of pantoprazole sodium to patients with liver cirrhosis classified as Child- Pugh A and B, serum elimination half-lives of pantoprazole increased to between 3 and 6 hours (pantoprazole 20 mg tablet) or 7 to 9 hours (pantoprazole 40 mg tablet and powder) and AUC values increased by a factor of 3 to 5 (pantoprazole 20 mg tablet) or 5 to 7-fold (pantoprazole 40 mg tablet and powder). Maximum serum concentrations, C_{max}, in these patients increased only slightly (1.3-fold after oral administration, 1.5-fold after I.V. application) relative to healthy subjects. The observed pharmacokinetic changes did not lead to relevant accumulation following once-daily dosing.

Age, Gender, Race

As with other clinically used PPIs, a small percentage of the population (about 3% Caucasians, 20% Asians) shows slower elimination of pantoprazole (T_{1/2} being up to 10 hours as compared with 1 hour). Such persons are known as poor metabolizers as a result of a deficiency of the CYP2C19 enzyme. In these individuals the metabolism of pantoprazole is probably mainly catalyzed by CYP3A4. After a single-dose administration of 40 mg pantoprazole, the mean area under the plasma concentration- time curve was approximately 6 times higher in poor metabolizers than in subjects having a functional CYP2C19 enzyme (extensive metabolizers). Mean peak plasma concentrations were increased by about 60%. These findings have no implications for the posology of

pantoprazole.

Results from several studies in children/adolescents from birth to 16 years indicate that the pharmacokinetics of pantoprazole is similar to those in adults when appropriately adjusted by patient weight, despite somewhat decreased clearance in patients less than 1 year old. Similar to adults, pediatric patients who were poor metabolizers of CYP2C19, exhibited reduced clearance that was more than 70% lower than the typical value. Compared with younger subjects, slight increases in AUC and C_{max} were noted after single and repeated oral administration of pantoprazole to healthy elderly subjects (age >65 years). However, no dose adjustment is necessary in elderly patients.

Drug Interactions

Pantoprazole is metabolized in the liver via the CYP enzyme system. An interaction of pantoprazole with other drugs or compounds, which are metabolized using the same enzyme system, cannot be ruled out. Nevertheless, in specific tests pantoprazole did not affect the clearance of several compounds metabolized by CYP enzymes. Vice-versa, all drugs that were tested regarding their potential influence on the pharmacokinetics of pantoprazole had no relevant effect.

No detectable interactions between pantoprazole and any other commonly prescribed co-medication tested so far were found.

Metabolism of pantoprazole occurs via oxidation by the CYP enzyme system, predominantly by CYP2C19 and CYP3A4. Interaction studies with drugs also metabolized by these pathways, like carbamazepine, diazepam, glibenclamide, nifedipine, phenytoin, and an oral contraceptive containing levonorgestrel and ethinyl estradiol did not reveal clinically significant interactions. Results from a range of interaction studies demonstrate that pantoprazole does not affect the metabolism of active substances metabolized by CYP1A2 (such as caffeine, theophylline), CYP2C9 (such as piroxicam, diclofenac, naproxen), CYP2D6 (such as metoprolol), or CYP2E1 (such as ethanol) and does not interfere with p-glycoprotein related absorption of digoxin. There were no interactions with concomitantly administered antacids. Interaction studies have also been performed administering pantoprazole concomitantly with the respective antibiotics (clarithromycin, metronidazole, amoxicillin). No clinically relevant interactions were found.

6. PHARMACEUTICAL PARTICULARS

6.1 List of excipients

Not applicable.

6.2 Incompatibilities

This medicinal product should not be manufactured or mixed with solvents other than those stated in sections 4.2 and 6.6.

6.3 Shelf life and Special Precautions for Storage

Primary Packaging Material	Shelf Life	Storage Condition
Powder		
Glass vials closed with rubber stoppers and sealed with aluminium caps.	24 months	Store below 30°C
Ready for use solution		
Glass vials closed with rubber stoppers and sealed with aluminium caps or the container of the solvent if used for infusion.	12 hours	Do not store above 25°C

6.4 Nature and Content of Container

Colourless type I glass vial, closed with a red rubber stopper and sealed by aluminium cap containing 40 mg powder for solution for injection.

Pack size of 1 vial and 10 vials.

Not all registered pack sizes are marketed.

6.5 Special Precautions for Disposal and Other Handlings

A ready-to-use solution is prepared by injecting 10 ml of sodium chloride 9 mg/ml (0.9%) solution for injection into the vial containing the powder. The appearance of the product after reconstitution is a colourless to faintly yellow solution.

This solution may be administered directly or may be administered after mixing it with 100 ml sodium chloride 9 mg/ml (0.9%) solution for injection or glucose 55 mg/ml (5%) solution for injection. Glass or plastic containers should be used for dilution.

After preparation, the solution must be used within 12 hours. From a microbiological point of view, the product should be used immediately.

After reconstitution, or reconstitution and dilution, chemical and physical in use stability has been demonstrated for 12 hours at not more than 25°C. If not used immediately, in-use storage times and conditions prior to use are the responsibility of the user.

Pantoprazole Sandoz 40 mg powder for solution for injection should not be prepared or mixed with solvents other than those stated. The medicine should be administered intravenously over 2-15 minutes.

The contents of the vial are for single use only. Any product that has remained in the container or the visual appearance of which has changed (e.g. if cloudiness or precipitation is observed) has to be disposed of in accordance with local requirements.

7. Product Registration Holder

Sandoz Products Malaysia Sdn. Bhd.
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47400 Petaling Jaya, Selangor, Malaysia.

8. Date of Revision of the Text

~~Mar 2024~~ Aug 2024